

SPECIALIZED MEDICAL CENTER & ASSOCIATES

Department of Orthopedics & Spinal Neurosurgery • 450 University Ave, Suite 300 • Phone: (555) 234-5678

Insurer: Central Health Insurance	Patient: Jane M. Doe
Appeals Dept: Appeals & Grievances Committee	Policy / ID: CHM-99482103-01
Claim / Case #: DEN-2026-8812-CL	Date of Birth: 04/14/1982
Target Procedure: CPT 72148 (Lumbar MRI without contrast)	Diagnoses: ICD-10 M54.5 (Low Back Pain), M51.1 (Radiculopathy)

SUBJECT: Formal First-Level Expedited Appeal regarding denial of coverage for *Lumbar Spine MRI without contrast (CPT: 72148)*.

1. STATEMENT OF DISPUTE & BASIS OF ADVERSE DETERMINATION

Acting as the authorized medical representative for **Jane M. Doe**, we formally request immediate reconsideration and complete overturn of the adverse coverage determination issued on **August 02, 2026**, which denied prior authorization for CPT 72148 citing *"Lack of medical necessity / Insufficient documentation of exhausted conservative therapy"*.

2. CLINICAL SUMMARY & MEDICAL NECESSITY

The patient presented with an 8-week history of incapacitating low back pain radiating to the right lower extremity along the L5 dermatome, accompanied by paresthesias and distal paresis (4/5 motor strength in ankle dorsiflexion). Physical examination confirms **positive Lasègue / Straight Leg Raise test at 35°** in the right lower extremity and diminished right Achilles deep tendon reflex (+/++++).

These objective clinical findings demonstrate acute lumbar nerve root compression with progressive neurological deficit, establishing the imperative medical necessity for diagnostic MRI to direct surgical decompression versus image-guided interventional management.

3. DOCUMENTED EVIDENCE OF CONSERVATIVE THERAPY COMPLIANCE

The insurer's assertion of unexhausted conservative management is clinically inaccurate. The enclosed contemporaneous clinical chart certifies strict completion of multi-modal conservative lines of care:

Period / Dates	Clinical Intervention	Outcome / Observations
06/12/2026 – 07/24/2026 (6 full consecutive weeks)	Physician-Supervised Physical Therapy 18 documented sessions at ActiveLife Rehabilitation.	Refractory course. Persistent visual analog pain score 8/10; antalgic gait claudication.
06/10/2026 – Present	First-Line Pharmacotherapy Prescription NSAID (Celecoxib 200mg/day) + Neuromodulator (Pregabalin 75mg BID).	Zero meaningful therapeutic relief of neuropathic radicular pain.
07/28/2026	Follow-up Neurological Assessment	Objective progressive sensory deficit across right L5 dermatomal territory.

4. REGULATORY GUIDELINES & CLINICAL COVERAGE CRITERIA (CMS LCD / NCD)

Pursuant to Centers for Medicare & Medicaid Services Local Coverage Determination (**CMS LCD L34212**) and National Coverage Determination (**NCD 220.4**) for Lumbar Spine MRI:

"Lumbar spine MRI without contrast is medically indicated and covered following at least 4 to 6 weeks of documented, supervised conservative management (physical therapy, medical therapy) in the presence of objective radicular signs, reflex asymmetry, or progressive neurological deficit."

Patient Jane M. Doe satisfies these criteria across all dimensions, having documented over 6 consecutive weeks of failed multimodal conservative therapy and active motor/sensory radicular compromise.

5. FORMAL DEMAND & CONCLUSION

We respectfully demand immediate overturn of this denial and prompt authorization for **CPT 72148** within 5 business days to prevent permanent, irreversible neurological impairment. We formally reserve all rights to pursue independent external medical review and regulatory remedies under federal and state guidelines.

Dr. Alexander Vance, MD

Spine & Orthopedic Surgery Specialist

NPI: 1982730491 • State License: 8492019

Jane M. Doe

Patient / Insured Member Signature of Authorization

Date: 08/15/2026